

Department of Obstetrics And Gynaecology
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 49.9 Yr/F
Admission No.: anonymous	Department Unit: Obstetrics And Gynaecology (Obs Unit 1)	Consultant/Faculty: anonymous
Ward: 1 B 2 Gynecology Ward	Room/Bed: 1 B 2 Gynecology Ward/ 1 B 2 Bed 17	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 31-Mar-2023 15:21	D.O.D. : 04-Apr-2023	D.O. Print Report: 08-Feb-2026 15:14

PROVISIONAL DIAGNOSIS:

48 YEARS/ P2L2/ BOTH LSCS/ TL DONE/ HYPOTHYROID/ USG S/O OVARIAN MASS/ CYSTADENOMA

FINAL DIAGNOSIS:

48 YEARS/ P2L2/ BOTH LSCS/ TL DONE/ USG S/O OVARIAN MASS/ CYSTADENOMA/ HYPOTHYROID

CHIEF COMPLAINTS:

ABDOMINAL DISTENSION SINCE 2 YEARS

PAIN ABDOMEN SINCE 2 YEARS

HISTORY OF PRESENT ILLNESS: 48 YEARS/ P2L2/ BOTH LSCS/ TL DONE/ POSTMENOPAUSAL/ HYPOTHYROID. HER USG WAS S/O OVARIAN MASS/ CYSTADENOMA. HENCE SHE WAS ADMITTED FOR FURTHER MANAGEMENT. HER VITALS WERE STABLE AT THE TIME OF ADMISSION.

GENERAL EXAMINATION:

GC - FAIR

PR- 76/MIN, RR-20/MIN, BP-130/100 MMHG

TEMP-AFEBRILE

JVP: NOT ELEVATED

PALLOR/ICTERUS/CYANOSIS/CLUBBING/EDEMA ABSENT

SYSTEMIC EXAMINATION:

CVS/CNS/RS-WNL

PA: ABDOMEN OVERLY DISTENDED, APPEARS APPROXIMATELY 34 WEEKS IN SIZE.

PS: OS VISUALISED, PINPOINT. CERVIX POSTERIOR. ACTIVE WHITE CURDY DISCHARGE PRESENT. VULVA AND VAGINA APPEARS NORMAL.

PV: OS PINPOINT. CERVIX POSTERIOR, FIRM IN CONSISTENCY, UTERUS COULD NOT BE PALPATED. A 25X25 CM MASS FELT ON RIGHT SIDE, SEPARATE FROM UTERUS SIDE TO SIDE MOBILE, NON TENDER. B/L FORNICES AND POD FREE AND NON TENDER

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
01-Apr-2023 15:16	FASTING GLUCOSE	Fasting Blood Sugar	96	70 - 110 mg/dl
	HBsAg (Rapid test)	Interpretation	Negative	-
		Result	Refer Reports*	-
	HCV Antibody (Rapid)	Interpretation	Negative	-

test)

	Result	Refer Reports*	
PP GLUCOSE	Glucose P P	95	< 140 mg/dl
URINE ROUTINE AND MICROSCOPIC EXAMINATION (R/M)	Appearance	CLEAR	-
	Bilirubin	NOT DETECTED	-
	Casts	NOT DETECTED	-
	Colour	PALE YELLOW	-
	Comments	Refer Reports*	-
	Crystals	NOT DETECTED	-
	Epithelial Cells	5-10	0 - 5 / hpf
	Glucose	NOT DETECTED	-
	Ketones	NOT DETECTED	-
	Nitrite	NEGATIVE	-
	Others	NOT DETECTED	-
	Protein	NOT DETECTED	-
	Pus cells	OCCASIONAL	-
	RBC	NOT DETECTED	-
	Reaction pH	6.0	5 - 7 .
	Specific Gravity	1.020	1.003 - 1.035 .
	Urobilinogen	NORMAL	-
31-Mar-2023 17:32	Liver Fuction Test (Lft)	Albumin	4.1
		ALP	50
		ALT	16
		AST	22
		Direct Bilirubin	0.48
		globulin	2.7
		Indirect Bilirubin	2.15
		Total Bilirubin	2.63
		Total Protein	6.8
	Rft	Chloride	105
		Creatinine	0.90
		Potassium	3.67
		Sodium	137
		Urea	20
31-Mar-2023 17:23	ACTIVATED PARTIAL THROMBOPLASTIN TIME (APTT)	APTT HEADER	22.7
		APTT NOTE	Refer Reports*
		clinical details	--

BLOOD GROUPING	Blood Group	B	-
ABO and RhD	Note	Refer Reports*	-
	RH Type	Positive	-
COMPLETE BLOOD	Comments on PBS	Refer Reports*	-
COUNT (CBC)	Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
	Absolute Eosinophil Count	0.15	0.02 - 0.5 x10 ³ /μL
	Absolute Lymphocyte Count	1.96	1 - 3 x10 ³ /μL
	Absolute Monocyte Count	0.33	0.2 - 1.0 x10 ³ /μL
	Absolute Neutrophil Count	4.77	2 - 7 x10 ³ /μL
	Basophil	0.1	00 - 01 %
	Eosinophils	2.1	01 - 06 %
	HCT	30.3	36 - 46 %
	Hemoglobin	9.1	12 - 15 g/dl
	Lymphocytes	27.1	20 - 40 %
	MCH	22.6	27 - 32 pg
	MCHC	30.0	31.5 - 34.5 g/dL
	MCV	75.4	83 - 101 fL
	Monocytes	4.6	2 - 10 %
	Neutrophils	66.1	40 - 80 %
	Platelet Count	141	150 - 410 x10 ³ /μL
	RBC	4.02	3.8 - 4.8 x10 ⁶ /μL
	Red Cell Distribution Width (RDW CV)	19.0	11.6 - 14.0 %
	Total Leukocyte Count(TLC)	7.22	4 - 10 x10 ³ /μL
LACTATE	LDH	328	230 - 460 u/l
DEHYDROGENASE (LDH)			
PROTHROMBIN TIME (PT INR)	clinical details	--	-
	International Normalised Ratio (INR)	1.1	-
	PT INR Header	10.9	09.6 - 12.2 sec
	PT INR Note	Refer Reports*	-

*Refer Lab Reports for details.

OTHER INVESTIGATIONS: BLOOD GROUP: B POSITIVE

SERUM INHIBIN B: 24.3 pg/mL

SERUM CEA: 2.26 ng/mL

SERUM TSH: 7.24 uIU/mL



C-DAC: Centre for Development of Advanced Computing
C-56/1, Sector-62, Noida - 201307, Uttar Pradesh,
91-0120-3063311-13.



SERUM AFP: 2.31 ng/mL

USG WHOLE ABDOMEN (24/02/23): MILD HEPATOSPLENOMEGALY, FINDINGS S/O OVARIAN MASS/ CYSTADENOMA, LEFT ECTOPIC/ RENAL AGENESIS.

HOSPITAL COURSE: 48 YEARS/ P2L2/ BOTH LSCS/ TL DONE/ POSTMENOPAUSAL/ HYPOTHYROID. HER USG WAS S/O OVARIAN MASS/ CYSTADENOMA. HENCE SHE WAS ADMITTED FOR FURTHER MANAGEMENT. HER VITALS WERE STABLE AT THE TIME OF ADMISSION. ALL RELEVANT INVESTIGATIONS SENT. PAC CLEARANCE TAKEN. ENDOMETRIAL BIOPSY WAS DONE ON 01/04/23. PATIENT CONDITION IS FAIR AT PRESENT AND HENCE BEING DISCHARGED WITH ADVICE TO FOLLOW UP IN GYNAEC OPD WITH EB REPORT AND CECT (A P) REPORT.

CONDITION AT DISCHARGE: GENERAL EXAMINATION: PR:80/MIN, BP: 130/90 MMHG, RR: 18/MIN, TEMP: AFEBRILE
PALLOR/ICTERUS/CYANOSIS/CLUBBING/LYMPHADENOPATHY: ABSENT; EDEMA: ABSENT

SYSTEMIC EXAMINATION: CNS/CVS/RS-WNL

P/A ABDOMEN OVERLY DISTENDED, APPROX. 34 WEEKS SIZE.

L/E NO ACTIVE BLEEDING PV

ADVICE ON DISCHARGE:

HIGH PROTEIN DIET

TAB. THYROXINE 75 MCG OD BBF

TAB. FE 1 TAB OD

TAB. CALCIUM VIT. D 1 TAB BD

TAN. MVBC 1 TAB OD

TAB. DROTIN SOS IF PAIN

TAB. CEFEXIME 200 MG BD FOR 1 DAY

TAB. PANTOP 40 MG BBF X 1 DAY

TAB. PAUSE MF 500 MCG SOSX 1 DAY

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 28-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : FOLLOW UP IN GYNAEC OPD WITH CECT ABDOMEN AND PELVIS REPORT ON MONDAY/FRIDAY OR SOS IN CASE OF SEVERE PAIN. REVIEW IN ENDOCRINE OPD IVO RAISED TSH.

Summary Prepared By

anonymous

Summary Approved By

anonymous